

HOW TO OPEN A HOME CARE ORGANIZATION IN VIRGINIA

A Virginia-licensed Home Care Organization — delivering non-medical personal care (hands-on help with the activities of daily living) under registered-nurse supervision, so seniors and adults with disabilities can live safely and independently at home.

A step-by-step startup and operations guide

Licensing (VDH OLC under 12VAC5-381) • Screening & training • Getting paid • Operations

Regulatory basis

The license is the **Home Care Organization (HCO)** license under 12VAC5-381 (Regulations for the Licensure of Home Care Organizations), adopted under the Code of Virginia § 32.1-162.7 et seq. (Article 7.1). Licensed and overseen by the Virginia Department of Health (VDH), Office of Licensure and Certification (OLC), Home Care and Hospice Unit. This is a NON-MEDICAL provider — hands-on personal care with the activities of daily living, under registered-nurse supervision (Part V of the regulation) — not skilled home health, which is separately authorized. In Virginia it is this hands-on personal care that requires the license; a provider of ONLY homemaker, chore, or companion services is exempt. No Certificate of Public Need is required for home care personal care. Funding: private pay; Virginia Medicaid personal care through DMAS and Cardinal Care, including the CCC Plus waiver; Veterans' benefits; and long-term-care insurance.

How to Use This Guide

This guide walks you from “I want to start a home care business” to “I am licensed, serving clients, and getting paid” in Virginia. The big picture is simple: in Virginia, non-medical personal care is delivered by a Home Care Organization licensed by the VDH Office of Licensure and Certification under 12VAC5-381. Unlike some states, Virginia does NOT require a Certificate of Public Need for home care — entry is open to qualified applicants, which makes it a faster, lower-cost way into the state’s senior-care market. Your agency provides real hands-on personal care — help with bathing, dressing, toileting, transferring, and eating — alongside the related homemaking that supports it. Skilled nursing (administering medications, injections, wound care) stays outside your scope; that is home health, a separate and heavier authorization.

Read Parts 1 through 4 before you spend real money — they tell you what you are allowed to do and exactly how to get licensed, including Virginia’s distinctive scope line and its registered-nurse supervision requirement. Parts 5 and 6 are about revenue: where clients come from and how you get paid, including Virginia Medicaid’s personal care through Cardinal Care and its Provider Choice Electronic Visit Verification. Parts 7 through 10 keep you safe and compliant once you are open. The two worked examples show how it comes together in a metro market, and the contacts, glossary, and references at the end are your quick-reference desk. Verify every fee, figure, and phone number before you rely on it — requirements change.

The one-paragraph version

Form your business with the Virginia State Corporation Commission and get an EIN, insurance, and a surety or fidelity bond. Confirm your scope (hands-on personal care requires the license; companion-only is exempt). Line up a registered nurse to assess clients, write each plan of services, and supervise personal care with a visit at least every 90 days. Apply for a Home Care Organization license through the VDH OLC Portal at least 60 days before opening, pay the \$1,500 triennial fee, and show three months’ reserve operating funds. Run a Virginia State Police criminal record report and a sworn disclosure on your administrator, owners, and every caregiver — checked against the barrier-crimes list — before client contact, and screen everyone against the exclusion lists. Pass your OLC survey. Bill private-pay clients from day one; to bill Medicaid, enroll through the DMAS PRSS provider portal, contract with Cardinal Care managed care organizations, and capture every visit through a DMAS-compliant EVV system. Stay in scope, supervise every client at least every 90 days, and document everything.

The Path at a Glance

1. Form your business — register your LLC or corporation with the Virginia State Corporation Commission, get your EIN, open a business bank account, and put insurance and a surety/fidelity bond in place.
2. Confirm your scope and service area — hands-on personal care (which requires the license) and the counties you will cover; a service that does only homemaker, chore, or companion work is exempt from licensure.
3. Line up your registered nurse — an RN to assess each client, write the plan of services, and provide supervision with a visit at least every 90 days.
4. Prepare policies and records — a 12VAC5-381-compliant policy manual, client and personnel records, and your administrative records.
5. Screen your people — a Virginia State Police criminal record report and a sworn disclosure (checked against the barrier-crimes list) plus exclusion screening, before any client contact.
6. Apply to the OLC and pass your survey — submit through the OLC Portal at least 60 days before opening with the \$1,500 triennial fee and proof of three-month reserve operating funds, then pass the OLC survey.
7. Start serving private-pay clients — begin revenue immediately while you build Medicaid capability.
8. Add Medicaid — enroll through the DMAS PRSS provider portal, contract with Cardinal Care managed care organizations, and set up your Provider Choice EVV system.
9. Operate and stay compliant — supervise every client at least every 90 days, keep screening and training current, and document everything.
10. Grow — deepen referral relationships, recruit and retain good caregivers, and expand only as fast as your RN supervision can follow.

Part 1 — What This Business Is

In Virginia, a **Home Care Organization** is a public or private entity that provides an organized program of home care services — which may include home health, personal care, or pharmaceutical services — in a person's residence. Your non-medical business provides **personal care**: hands-on assistance with the activities of daily living, under registered-nurse supervision, and leaves skilled nursing and certified home health out. You are licensed and surveyed by the VDH Office of Licensure and Certification (OLC), under 12VAC5-381 and Code of Virginia § 32.1-162.7 et seq.

The most important structural fact for a new owner: a home care organization does NOT require a Certificate of Public Need. In Virginia, entry into home care is open to qualified applicants — you meet the licensure standards and pass the survey; you do not have to prove community need or win a competitive review. That makes home care a far faster, lower-cost way into the senior-care market than facility-based care.

The non-medical line — know it cold

Your home attendants may help with bathing, dressing, mobility, transferring, toileting, eating, and the related homemaking, and may *assist a client with self-administration* of medication (reminders, bringing the container, reading the label). They may NOT administer medications, give injections, or do wound care — those are skilled nursing tasks that require a home health authorization. Cross that line and you are operating outside your license.

The Virginia scope line is the concept to understand from day one. It is the moment your staff put hands on a client for ADL help that triggers the license. A provider of only homemaker, chore, or companion services — nonmedical care, supervision, and socialization with no hands-on care — is exempt from licensure under 12VAC5-381-30. But most agencies that want Medicaid clients, or that serve people who genuinely need help at home, provide hands-on personal care, and therefore hold the license. Decide honestly which side of the line you are on, and if you provide personal care, get licensed.

It helps to understand why the market is strong. Virginia is aging steadily — the population age 60 and older has grown past 1.9 million and continues to climb — and the overwhelming preference among older adults is to stay in their own homes rather than move to a facility. That preference, combined with a Medicaid program that funds personal care at home through Cardinal Care and a documented shortage of home-care workers, means demand for reliable non-medical personal care is high and rising across the state's metros and rural counties alike. A well-run agency that screens carefully, supervises on schedule, and shows up reliably is filling a real and durable need.

Part 2 — Getting Licensed with the OLC

You apply for your Home Care Organization license through the **VDH OLC Portal** — the Office of Licensure and Certification's online licensing system — and the Home Care and Hospice Unit within the Division of Acute Care Services handles HCO licensure and inspection (Home Care Organization unit line: 804-367-2104). Submit a complete initial application at least 60 days before your planned opening date; an incomplete application goes inactive after six months, and you must reapply.

- **The fee.** The triennial fee for all Home Care Organization licenses is \$1,500 (effective 05/22/2025 — verify the current amount). The license is renewed every three years.
- **Reserve operating funds.** Initial applicants must show proof of funds sufficient to operate the organization for the three-month period after the license is issued (§ 32.1-162.9).
- **Prelicensure consultation.** The OLC offers prelicensure consultation and technical assistance — use it to understand the regulation and the survey before you apply.
- **The survey.** Your activities and services are subject to OLC inspection for compliance with 12VAC5-381. Walk your own operation against the regulation before you invite the surveyor, and fix gaps in advance.

A note on the fee and timing

Fees and timelines change. The \$1,500 triennial fee reflects the schedule effective in 2025; confirm the current fee and the current processing timeline directly with the OLC before you budget. And remember the 60-day rule — your complete application must be on file at least 60 days before you plan to open, so work backward from your target date.

What does the OLC look for at the survey? In broad terms: that your policies are written and implemented, not just sitting in a binder; that your client records contain signed plans of services, assessments, and evidence of RN supervisory visits; that your personnel records show the completed Virginia State Police criminal record report, the sworn disclosure, exclusion screening, documented competency, and health screening; and that you maintain the administrative records the rules require. Prepare against the regulation, and the survey confirms what you already do rather than catching you out.

Plan your timeline realistically. Business formation and insurance take a week or two. Writing or adopting a compliant policy manual, building your record systems, and securing your registered nurse take longer, and screening and hiring your first caregivers run in parallel. Because the application must be in at least 60 days before opening, start early and keep momentum.

Accreditation is optional

You are not required to be accredited to hold a Virginia Home Care Organization license. Some agencies pursue accreditation later — when payers or referral partners value it — but it adds cost and is not part of the non-medical licensure path. Start with the license; consider accreditation only if and when your business model calls for it.

Part 3 — Background Screening & Caregiver Qualifications

Before anyone has contact with a client, Virginia requires you to obtain a **criminal record report from the Virginia Department of State Police** for every applicant for compensated employment, within 30 days of employment, together with a **sworn disclosure statement** of criminal convictions and pending charges. You must ensure no employee has been convicted of a **barrier crime** under § 32.1-162.9:1 — a barrier-crime conviction bars employment. The report must not be dated more than 90 days before the date of employment.

The rule people miss — no unsupervised contact until the report is back

No employee works in a position involving direct client contact until you have received the original criminal record report — unless that person works under the *direct supervision* of another employee whose background check is already complete. A criminal record report stays valid while the employee remains in continuous service; a new report and sworn statement are required when someone moves from one home care organization to another. Report any positive drug-test result to the relevant health regulatory board as § 32.1-162.9:1 requires.

Beyond the criminal check, screen every employee, contractor, and vendor against the federal OIG List of Excluded Individuals/Entities, the SAM exclusions, and the Virginia Medicaid exclusion list before hire and monthly thereafter — Virginia will not license an operator sanctioned under the federal health care fraud statute (42 U.S.C. § 1320a-7b). Personnel records are kept confidential, with health information separate, and retained at least three years after termination.

On training and competency: before client contact, each home attendant completes orientation — the role and the non-medical scope, client rights and confidentiality, recognizing and reporting abuse and neglect, assisting with the activities of daily living, infection control, safety and emergency procedures, and the medication boundary — and the registered nurse verifies competency before independent assignment. Personal care aides at a VDH-licensed agency receive at least 12 documented hours of agency-provided training each year, on top of their initial preparation.

Who leads the agency: you designate a qualified Administrator responsible for day-to-day management, with the training and experience the regulation requires, and a qualified alternate for when the Administrator is away, so supervision and availability never lapse. On employment and pay: as a third-party employer you follow the federal Fair Labor Standards Act and cannot claim the companionship exemption for your caregivers — pay at least Virginia's minimum wage (which is CPI-indexed — \$12.77 in 2026) and never less than the federal minimum, pay overtime after 40 hours in a workweek, and track compensable travel between clients. Classify workers correctly as employees, not contractors; misclassification is a common and costly mistake.

Part 4 — Setting Up to Operate

Form your entity with the Virginia State Corporation Commission (scc.virginia.gov), get your EIN, open a business bank account, and put insurance in place — general and professional liability, a surety or fidelity bond (or third-party crime insurance) that protects clients against loss from employee theft or fraud, and workers' compensation. Virginia requires workers' compensation coverage once you regularly employ more than two employees; the Virginia Workers' Compensation Commission administers it.

The 90-day supervision clock

For every client receiving personal care, your registered nurse must make a supervisory visit to the client's home as often as the client's needs require — and in no case less often than every 90 days — reviewing the client's condition, the plan of services, and the attendant's performance. Build a tracker from each client's start date so no 90-day visit is ever missed; it is the single thing surveyors check most closely.

Your registered nurse is central to a compliant operation even though you are non-medical. The RN conducts the assessment that determines each client's needs, develops the written plan of services (in collaboration with the client and family) that the client receives and the assigned attendant reviews before services begin, and makes the

supervisory visits. You do not need the nurse on-site all day, but you do need a real, reachable professional standing behind the care — so line this up before you take your first client.

Build your record-keeping to survive a survey. For each client: the assessment, the written plan of services and its reviews, service (visit) documentation that matches the plan, RN supervisory-visit records, and any incident, change-of-condition, or grievance records. For each employee: the criminal record report and sworn disclosure, exclusion screening, orientation and RN-verified competency, health screening, driver checks where relevant, and the annual in-service record. For the agency: your policies, administrative records, and (for Medicaid) enrollment and EVV setup. Retain personnel records at least three years after termination. Good records are how you prove, on any given day, that the care was appropriate and delivered.

Part 5 — Finding Clients & Referrals

For many clients, the entry point to in-home services is the aging network. The national Eldercare Locator (1-800-677-1116) routes callers to their local Area Agency on Aging, and Virginia's Department for Aging and Rehabilitative Services (DARS) and the local departments of social services are central to how older adults find help at home. Introduce yourself to the Area Agency on Aging that serves your counties early.

Referral targets are in your Referral System edition

The Virginia edition of the Referral System lists real hospitals and discharge planners, skilled-nursing and rehab centers, hospices, assisted living communities, funeral homes, elder-law attorneys, and aging-services and care-management partners — by metro (Northern Virginia, Richmond, Virginia Beach, and Roanoke), with names and phone numbers — so you can start outreach immediately. Verify each contact before you rely on it.

Beyond the aging network, your steadiest referral sources are hospital and skilled-nursing discharge planners, geriatric care managers, senior-living communities, and elder-law attorneys — the people families turn to at the moment they realize they need help at home. The reason referrals matter so much is that home care is a trust purchase made at a stressful moment — a hospital discharge, a fall, a diagnosis, a family caregiver burning out. At that moment, people ask someone they trust, and the agency that professional names is the one that gets the client. Your growth job, especially early, is to earn a place on a small number of trusted referral lists and then deliver so reliably that those sources keep sending clients. One discharge planner or care manager who trusts you can be worth more than a year's advertising.

Private-pay clients often come from a different direction — adult children researching options online, senior-living communities whose residents need extra in-home support, and word of mouth from satisfied families. A clear website, a Google Business Profile, and a simple one-page leave-behind for discharge planners and care managers go a long way. Whatever the channel, your reputation for reliability is your best marketing. Build every referral relationship lawfully — see the anti-kickback note in Part 6.

Part 6 — Getting Paid

You can serve private-pay clients from the day your license is issued — many agencies start there. To bill Medicaid, enroll as a provider through the DMAS **Medicaid Enterprise System (MES) / PRSS provider portal** (Provider Enrollment Helpdesk 804-270-5105 or 888-829-5373), using your NPI; you revalidate at least every five years. Virginia Medicaid's community personal care flows mostly through **Cardinal Care** managed care and the CCC Plus waiver, which has no waiting list. Enrolling does not by itself generate clients — you must contract or coordinate with the Cardinal Care managed care organizations to serve their members.

You bill agency-directed personal care (and respite and companion) under the correct codes and modifiers; consumer-directed services are handled through a fiscal agent and cannot be billed by an agency. Service authorization for fee-for-service runs through Acentra Health; for managed-care members, the Cardinal Care MCO care coordinator authorizes the service. Personal-care hours are individually assessed and authorized per client, so there is no single flat rate — but the procedure codes are standard, and the current rates are published by DMAS.

The billing codes you'll use

These are the Virginia Medicaid procedure codes for the services a Home Care Organization provides, drawn from DMAS's published personal-care rate documents. Confirm the exact code, modifier, and rate for your service and payer on the DMAS rate-setting page before you bill — codes and rates are updated periodically.

Service (agency-directed)	Procedure code(s)	Notes
Personal care	T1019 (per 15 min); S5125	Your core service; billed in whole hours under CCC Plus (see the whole-hours rule below)
Respite care (unskilled)	S5150 (per 15 min); S5151 (per diem)	Relief for the family caregiver; separately authorized; you

Service (agency-directed)	Procedure code(s)	Notes
		cannot bill respite for the same hours as personal care
Companion care	S5135	Non-hands-on supervision and socialization
RN/LPN supervisory visit	DMAS-99 (form)	Documents the required supervisory visit for personal care and respite

The consumer-directed equivalents (personal care S5126, respite T1005, companion S5136) are billed through the fiscal/employer agent, not by your agency. **The whole-hours rule:** CCC Plus personal care is billed in whole hours only — you cannot bill 1.5 hours for a visit. The rounding is done monthly, not per visit: if the extra minutes across the calendar month total 30 or more, round up to the next whole hour; if less than 30, round down.

Rates — verify the current sheet (date-stamped)

DMAS published updated personal care, respite, and companion rates effective July 1, 2025 (SFY 2026), posted on the DMAS rate-setting page (dmas.virginia.gov/for-providers/rates-and-rate-setting/). A further 20% increase to agency- and consumer-directed personal care, respite, and companion rates was authorized effective July 1, 2026 (with a required \$20/hour pass-through to workers) — confirm whether it took effect, because budget items can change. Do not budget from third-party rate figures; pull the current sheet from DMAS. Provider rate helpline: 800-552-8627.

One more billing reality: you must be enrolled in the DMAS PRSS portal AND contracted with each Cardinal Care managed care organization separately — under federal rules an MCO cannot pay a network provider who is not also enrolled in PRSS. Cardinal Care currently has five MCOs (Aetna Better Health, Anthem HealthKeepers Plus, Humana Healthy Horizons, Sentara, and UnitedHealthcare Community Plan — verify the current list), and each has its own claims pathway, so keep your PRSS enrollment and license information current to avoid payment delays from both DMAS and the plans.

Electronic Visit Verification — Provider Choice

Virginia requires Electronic Visit Verification for Medicaid personal care, respite, and companion visits, and it is a Provider Choice state: you select any DMAS-compliant EVV system — telephony, mobile app, or fixed device — that captures the six required data elements (the service, the individual receiving it, the individual providing it, the date, the location, and the start and end times). DMAS and the managed care organizations do not endorse a single vendor. Make sure your EVV data match the authorization and the claim before you bill; when they line up you get paid, and when they do not, claims pend or deny. Private-pay visits do not require EVV. Set this up and test it before you take your first Medicaid client, not after.

Your funding sources at a glance

Here is the honest map of who pays for non-medical personal care in Virginia. Because Medicaid personal-care hours are individually authorized, confirm current rates and authorized hours with DMAS and the payer rather than budgeting from any third-party figure.

Funding source	What it covers	How you access it
Private pay	Any non-medical personal care, no authorization needed — your fastest revenue	Your service agreement and schedule of charges; bill the client directly
Medicaid personal care (Cardinal Care)	Agency-directed personal care, respite, and companion for eligible members; hours individually authorized	Enroll in DMAS PRSS; contract with Cardinal Care MCOs; EVV required
CCC Plus waiver	Home & community-based services for elderly and disabled members at risk of nursing-facility care; no waitlist	Coordinate with the member's Cardinal Care care coordinator

Funding source	What it covers	How you access it
Veterans' benefits	In-home support for eligible veterans	VA programs and VA-contracted networks; verify eligibility with the VA
Long-term-care insurance	Non-medical home care where the policy covers it	Work with the client's policy; provide the documentation the insurer requires

The legal line on referrals

If any part of a client's care is paid by Medicaid, Medicare, or the VA, you may not pay for or reward referrals. The federal Anti-Kickback Statute makes it a crime to offer, pay, solicit, or receive anything of value to induce those referrals, and Virginia will not license an operator sanctioned under that law. Keep every referral relationship legitimate and for fair value — never a per-client fee.

Part 7 — Home, Fire & Safety

You provide care in clients' own homes, so most of your safety work is about the home environment and your attendants, not a facility. Cover these before and during service:

- A home safety assessment at intake and at each RN supervisory visit — fall risks, fire and electrical hazards, unsafe equipment, and blocked exits — with significant hazards escalated to the registered nurse and reflected in the plan of services.
- Working smoke alarms and a clear exit path in the areas where care is provided; advise the client and family on hazards you cannot fix.
- A simple emergency plan for each client: how to reach the agency, when to call 911 first, and what happens in severe weather or a power outage — Virginia's hazards include hurricanes and coastal storms, flooding, and severe winter weather.
- Infection-control supplies (gloves, hand sanitizer) and trained universal precautions for every attendant.
- Safe-transport rules if attendants drive clients — valid license, current insurance, a verified driving record, and seat belts.

Do I need a sprinkler system?

No. Because you deliver personal care in the **client's own home**, you are not operating a residential facility, so there is no facility sprinkler requirement for your agency. Your fire-safety duty is to assess each home for hazards, ensure working smoke alarms and clear exits where you provide care, and advise the client and family — not to install building fire-suppression systems.

Part 8 — Your First 90 Days Operating

Your first ninety days set the habits that keep you compliant for years. The goal is to open with a small number of clients served flawlessly rather than a large number served loosely — a clean early record is what carries you through your first survey and builds your referral reputation. Keep the order below, because assessment and the plan of services always precede service, and screening always precedes client contact.

1. Days 1–15: Finalize your policy manual, client and personnel record systems, and your 90-day supervisory-visit tracker. Confirm your RN is ready to assess and supervise, and confirm your OLC application is in (remember the 60-day lead).
2. Days 1–30: Complete the Virginia State Police criminal record report and sworn disclosure on all staff, run exclusion screening, and document orientation and RN-verified competency for every attendant before any client contact.
3. Days 15–45: Take your first private-pay clients. For each, complete the RN assessment, give the client the plan of services and review it with the attendant before services begin, and schedule the first 90-day supervisory visit.
4. Days 30–60: Enroll in the DMAS PRSS portal and begin Cardinal Care contracting; choose and test your Provider Choice EVV system before you rely on it.
5. Days 45–75: Build referral relationships — your Area Agency on Aging, hospital and skilled-nursing discharge planners, and the metro contacts in your Referral System edition.
6. Days 60–90: Run your first internal quality review — documentation completeness, supervisory-visit timeliness, and EVV capture — and fix any gaps before they become survey findings.

Part 9 — Common Mistakes to Avoid

Almost every problem that delays a license or triggers a survey finding traces back to a handful of avoidable mistakes. None are complicated; they are simply the things that slip when an owner is moving fast. Read this list before you open and again before each survey.

The single most common finding

If there is one thing surveyors see most often, it is a missed or late supervisory visit. The 90-day clock runs from each client's start date, every client is on their own schedule, and it is easy to lose track once you have more than a few. A simple tracker — client, start date, next-visit-due date — that your RN reviews weekly prevents the single most common deficiency in the program.

- Providing hands-on personal care while calling yourself “companion-only.” The moment you help with ADLs, you need the license.
- Missing the 90-day RN supervisory visit. Track it from each client's start date.
- Starting a client before the plan of services is in place and reviewed with the attendant.
- Letting a caregiver start on a pending criminal record report without the required direct supervision.
- Crossing the non-medical line — administering medication, giving injections, or doing wound care.
- Skipping monthly exclusion screening. A single excluded employee can jeopardize your Medicaid participation.
- Billing Medicaid without matching EVV, or billing consumer-directed codes as an agency — both cause denials.
- Under-pricing in Northern Virginia, where wages run well above the state minimum.

Part 10 — Staying Compliant Year-Round

- Make the RN supervisory home visit to every personal-care client at least every 90 days, and document each one.
- Keep the Virginia State Police criminal record report, sworn disclosure, and exclusion screening current; never let a new hire start client contact on a pending check without the required direct supervision.
- Renew your Home Care Organization license every three years with the \$1,500 fee, and keep your administrative records current — retain personnel records at least three years after termination.
- Keep attendant documentation and EVV records consistent for every Medicaid client, and resolve visit exceptions before you bill.
- Run regular internal quality reviews and act on what they show — documentation, supervision timeliness, incidents, and grievances.
- Report suspected abuse, neglect, or exploitation of an adult (age 60 or older, or an incapacitated adult) to Virginia Adult Protective Services at 1-888-832-3858 or the local department of social services; call 911 if the person is in immediate danger.

Compliance is not a once-every-three-years scramble before renewal — it is the daily rhythm of a well-run agency, and it is your best protection. An agency that supervises on schedule, screens before hire, keeps documentation current, and stays strictly in scope has little to fear from a survey, because the survey simply confirms what the agency already does. Treat every deficiency, complaint, or near-miss as information: find the root cause, fix it through your quality program, and adjust your training so it does not recur. Keep learning, too — Virginia updates its rules, DMAS issues bulletins, and payers change requirements — so revisit your policy manual at least annually and whenever a rule changes.

Two Worked Examples

The two examples below show the same path playing out in two different Virginia markets. They are illustrative — the people are invented and every phone number should be verified before you rely on it — but the sequence and the local contacts are real and current. Read them side by side to see what is universal (the license, screening, RN supervision, and EVV) and what is local (your Area Agency on Aging and your referral partners).

Worked Example 1 — Northern Virginia (Fairfax / Arlington)

Ana opens a personal care Home Care Organization in Fairfax, serving an affluent, high-cost, federally-employed market. Her path:

1. Entity & setup: She forms an LLC with the Virginia State Corporation Commission, gets her EIN, and lines up general/professional liability, a surety/fidelity bond, and workers' compensation (she'll pass the more-than-two-employee threshold quickly).
2. Scope & RN: Hands-on personal care, so she secures a registered nurse to assess clients, write each plan of services, and make the 90-day supervisory visits.
3. Licensing: She submits her HCO application through the OLC Portal at least 60 days ahead with the \$1,500 fee and three-month reserve funds, runs the Virginia State Police criminal record report and sworn disclosure on herself and her caregivers, and passes the OLC survey.
4. Referrals: She connects with the Fairfax Area Agency on Aging (703-324-5411) and discharge planners at Virginia Hospital Center (703-558-5000) and Inova Fairfax Hospital (703-776-4001), plus rehab partners (Fairfax Rehabilitation & Nursing 703-273-7705), hospice (VITAS 703-270-4300), and elder-law attorneys (Farr Law Firm 703-691-1888) from her Referral System edition.
5. Getting paid: She prices private-pay for the high-cost market, then enrolls in DMAS PRSS, contracts with Cardinal Care plans, and sets up a Provider Choice EVV system.

Worked Example 2 — Richmond

Marcus opens in Richmond, a mixed urban-suburban market with a large Medicaid population and a strong hospital system. His steps mirror Ana's, tuned to his region:

1. Entity & setup: LLC with the Virginia State Corporation Commission, EIN, insurance, surety/fidelity bond, and workers' comp.
2. Scope, RN & licensing: Same path — personal care under RN supervision, application through the OLC Portal 60 days ahead, State Police criminal record report and sworn disclosure, and the OLC survey.
3. Referrals: He works with the Dept. for Aging & Rehabilitative Services (DARS, 804-588-3333) and discharge planners at VCU Medical Center (804-828-9000), plus rehab (Southampton Rehabilitation & Healthcare 804-320-7901), hospice (Hospice of Virginia 804-281-0451), and elder-law attorneys (Lee Law Office 804-204-1500) from his Referral System edition.
4. Getting paid: He emphasizes Cardinal Care and CCC Plus waiver clients, keeps his RN caseload tight for 90-day supervision, then enrolls in DMAS PRSS and sets up EVV.

Notice what stays the same across both metros: the license (12VAC5-381), the screening (Virginia State Police criminal record report + barrier crimes), the RN plan of services and the 90-day supervisory visit, and Provider Choice EVV. Only the local Area Agency on Aging and referral partners change.

How to find your own county's numbers

Wherever you open, assemble your local list the same way. Start with the Eldercare Locator (1-800-677-1116) to reach your county's Area Agency on Aging. Then identify the hospitals and skilled-nursing facilities in your service area and ask each for the case-management or discharge-planning department. Find your local department of social services for both Adult Protective Services reporting and local aging services. Finally, introduce yourself to the geriatric care managers, senior-placement professionals, and elder-law attorneys in your area — they meet families

at the exact moment a home-care decision is being made. Your Referral System edition gives you a verified starting list for the four largest metros; build out from there.

Part 11 — Growing From Here

Once you are licensed, serving clients, and getting paid, growth is mostly a matter of doing more of what already works — deepening referral relationships, hiring and retaining good caregivers, and keeping your compliance airtight so nothing slows you down.

Grow in steps you can supervise. Because every personal-care client needs an RN supervisory visit at least every 90 days, your nurse's caseload is a real constraint — add clients and, eventually, additional staff or a second nurse in step with your ability to supervise, not faster than your quality can follow. If you expand your service area, make sure your RN can still reach every client's home on schedule.

Invest in caregiver recruitment and retention early, because in a tight labor market your ability to staff cases reliably is your real constraint on growth. Agencies that treat caregivers well — fair pay, respect, good scheduling, and real support — keep them, and a stable workforce is what lets you say yes when a discharge planner calls with a new client. Some owners eventually add a skilled home health line, which is a bigger, clinical business with a much heavier regulatory load; it is not necessary, and many successful agencies stay non-medical by choice. Whatever pace you set, the non-medical Home Care Organization you build now is a real, durable business serving a growing need.

Who to Call & What to Ask

For...	Who to contact
Your license (rules, application, survey)	VDH Office of Licensure and Certification — Home Care Organization unit 804-367-2104; apply through the OLC Portal. Ask: current fee, the 60-day application timing, and can I schedule a prelicensure consultation?
The regulation & scope	12VAC5-381; § 32.1-162.7 et seq. Ask: which parts apply to a personal-care-only agency (Parts I, II, V), and exactly what separates licensable personal care from exempt companion services?
Background checks	Virginia Department of State Police (criminal record report). Ask: how to request reports on applicants, the turnaround, and how to confirm a report is clear of barrier crimes under § 32.1-162.9:1.
Medicaid enrollment	DMAS Provider Enrollment Helpdesk 804-270-5105 / 888-829-5373; enroll in the MES/PRSS portal. Ask: how to enroll as an agency-directed personal care provider and the current requirements.
Cardinal Care plans	The managed care organizations that authorize and pay most personal care. Ask: how to contract as a network provider, and your authorization and billing rules for agency-directed personal care.
EVV	DMAS EVV team. Ask: the six required data elements, and how to confirm my chosen Provider Choice system meets DMAS requirements.
Report abuse of an adult	Virginia Adult Protective Services 1-888-832-3858 or the local department of social services; call 911 if the person is in immediate danger.
Business registration	Virginia State Corporation Commission (scc.virginia.gov); IRS for an EIN. Ask: how to register and stay in good standing so licensing and Medicaid enrollment are not held up.
Workers' compensation	Virginia Workers' Compensation Commission (workcomp.virginia.gov). Ask: the coverage requirement at more than two employees and how officer inclusion works.
Statewide aging gateway / referrals	Eldercare Locator 1-800-677-1116. Ask for your local Area Agency on Aging and aging

For...	Who to contact
	resources.

Key Terms at a Glance

- **Home Care Organization (HCO) license:** the VDH OLC license under 12VAC5-381 required to provide hands-on personal care.
- **Certificate of Public Need:** NOT required for home care personal care in Virginia — entry is open.
- **Personal care:** hands-on assistance with the activities of daily living under an RN plan of services — the licensable service.
- **Companion / homemaker / chore:** nonmedical help with no hands-on care — exempt from licensure if provided alone.
- **Plan of services:** the RN-developed written plan for each client, reviewed with the attendant and supervised at least every 90 days.
- **Barrier crime:** an offense in § 32.1-162.9:1 that automatically bars employment with a home care organization.
- **DMAS / Cardinal Care:** Virginia's Medicaid agency and its managed care program (including the CCC Plus waiver population).
- **EVV / Provider Choice:** Electronic Visit Verification; Virginia lets each provider choose a DMAS-compliant system.

Core Rules & References

- 12VAC5-381 — Regulations for the Licensure of Home Care Organizations (Parts I, II, and V for personal care)
- Code of Virginia § 32.1-162.7 et seq. — Home Care Organization licensing (Article 7.1)
- 12VAC5-381-30 — exemption for homemaker, chore, or companion-only providers
- 12VAC5-381-360 — personal care services; RN supervision not to exceed 90 days
- 12VAC5-381-110 & § 32.1-162.9:1 — criminal record report, sworn disclosure, and barrier crimes
- § 32.1-162.9 — the three-month reserve operating funds requirement
- 12VAC30-120 — Virginia Medicaid home and community-based services (Cardinal Care, CCC Plus waiver, aide training)
- 21st Century Cures Act § 12006 — the federal EVV mandate (Virginia Provider Choice model to DMAS)
- § 63.2-1606 — reporting adult abuse, neglect, and exploitation (Virginia APS)
- Title 65.2, Code of Virginia — the Virginia Workers' Compensation Act

This guide is a general educational resource for prospective Virginia home care organization owners. It is not legal, tax, or billing advice, and requirements, fees, and contacts change. Verify every step, figure, and phone number with the VDH Office of Licensure and Certification, DMAS, the Virginia Department of State Police, and the relevant payer before you rely on it.

Provisions cited reflect Virginia law and OLC guidance at the time of writing.